# 8.7 Hyperthyroidism and Thyroidectomy

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "I feel hot and nervous and lose weight even though I eat."<br>**Objective**<br>• HR 118/min<br>• Shaky<br>• Lost 5 kg<br>• TSH low, T4 high | **ACTUAL**<br>**Imbalanced nutrition: less than body requirements** related to increased metabolic rate as evidenced by weight loss despite good intake. | Within 4 weeks of treatment, weight steady. | • Give high-calorie, high-protein meals.<br>• Give antithyroid drugs and the beta-blocker as prescribed.<br>• Weigh once a week. | Weight steady: **goal met**. |
| **Subjective**<br>• After the thyroidectomy: "It hurts to swallow."<br>**Objective**<br>• Incision on the neck<br>• Voice hoarse | **ACTUAL**<br>**Acute pain** related to neck incision as evidenced by pain on swallowing. | Pain 3/10 or less within 24 hours. | • Sit her semi-Fowler with pillows supporting her neck.<br>• Give analgesia as prescribed.<br>• Offer cool, soft food.<br>• At first, check her voice every hour and report hoarseness that is getting worse. | Pain controlled: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Neck surgery close to the parathyroids and the airway | **RISK**<br>**Risk for injury** as evidenced by risk factors: neck hematoma, laryngeal nerve damage and hypocalcemia after thyroidectomy. | Airway clear and calcium normal. | • Keep a tracheostomy set at the bedside.<br>• Check the dressing and the back of the neck for bleeding.<br>• Report tingling around the mouth or in the fingers (hypocalcemia).<br>• Follow calcium. | No complications: **goal met**. |